

Michael T. Fillat, D.C. (TIN#: xx-xx81795)
10444 Clairemont Mesa Blvd
San Diego, CA 92124-1320
858-573-1104

Today's date: November 18, 2024

Patient: Abbie Gladstone #8774 DOB: 12/14/1999
Policy ID: 5571Q402J

Wednesday July 17, 2024 Provider: Michael Fillat D.C RE-EXAM

Date of service: 07/17/2024

NPI#: 1801824404

Electronically signed by Michael Fillat D.C on 07/17/2024 at 8:39am

Subjective

Rear-ended on July 11. Didn't feel too bad on that day but by later in day or next day she felt awful. She has LBP moving up to t/l spine and neck pain moving down to t/s. Also, frequent headache since. She went to MD and was given muscle relaxants and pain pills. They are not helping, but do help her sleep because they knock her out. She has tried to sleep without them and with them and more difficulty sleeping without them. She did hit her head hard on headrest. Was dazed but did not feel like she lost consciousness. The back of her head is still tender to the touch. Sitting semi-reclined, rest and heat seem to help with pain a little. Lying all the way down and any sudden or full movements make pain worse. No temporal as pain is constant. Overall she is not sleeping well. She has some general numbness and tingling in fingers and toes. She is allergic to penicillin. She takes ubrelvey for migraines, vitamin D and Mg. She had another auto accident last August. No fractures, surg, hosp, work inj. Eent, hla, bb wnl.

Complaints: **primary** -> bilateral **low back**, acute onset immediately at time of injury, pain described as: achy, dull, sharp, tender, intensity: moderate, level: 5/10, range: 5-7; **secondary** -> bilateral **c/t pain and headaches**, acute onset immediately at time of injury, pain described as: achy, dull, sharp, stiff, tender, intensity: moderate, level: 5/10, range: 5-7.

Objective

+percussion c2-4 and L2-5. -valsavas.

Pulses: left & right radial: strong & equal bilaterally 78 BPM.

Temperature: 97.8 degrees F.

Respiratory rate: 12 per minute.

Pulse Oximetry: 99%%.

Blood pressure: left arm: 122/76 mm/Hg.

Height: 64".

Weight: 112.

Smoking status: Never smoker.

Visual inspection of the patient revealed: -minors sign, no visible pain.

Mental status: Patient appeared: alert, awake, oriented to person, place, time and situation, able to show knowledge of current events, mildly confused.

Cervical ROM: flexion (active): WNL with pain; extension (active): WNL with pain; left rotation (active): WNL with pain; right rotation (active): WNL with pain; left lateral bending (active): WNL with pain; right

lateral bending (active): WNL with pain; flexion (passive): WNL with pain; extension (passive): WNL with pain; left rotation (passive): WNL with pain; right rotation (passive): WNL with pain; left lateral bending (passive): WNL with pain; right lateral bending (passive): WNL with pain.

Thoracic ROM: flexion (active): WNL with pain; extension (active): WNL with pain; left rotation (active): WNL with pain; right rotation (active): WNL with pain; left lateral bending (active): WNL with pain; right lateral bending (active): WNL with pain; flexion (passive): WNL with pain; extension (passive): WNL with pain; left rotation (passive): WNL with pain; right rotation (passive): WNL with pain; left lateral bending (passive): WNL with pain; right lateral bending (passive): WNL with pain.

Lumbar ROM: flexion (active): WNL with pain; extension (active): WNL with pain; left rotation (active): WNL with pain; right rotation (active): WNL with pain; left lateral bending (active): WNL with pain; right lateral bending (active): WNL with pain; flexion (passive): WNL with pain; extension (passive): WNL with pain; left rotation (passive): WNL with pain; right rotation (passive): WNL with pain; left lateral bending (passive): WNL with pain; right lateral bending (passive): WNL with pain.

Sensation: upper & lower extremity dermatomes: hypoesthesia light touch pin prick -> mostly c5-6 and L4-5.

Myofascial pain/involvement: left & right suboccipital: tightness, tenderness; left & right cervical paraspinal: tightness, tenderness; left & right thoracic paraspinal: tightness, tenderness; left & right thoracolumbar: tightness, tenderness; left & right lumbar paraspinal: tightness, tenderness.

Spinal tenderness on palpation: none; C2; C3; C4; L2; L3; L4; L5; left SI; right SI.

Reflexes: bilateral upper extremity, lower extremity 2+.

Orthopedic tests: Cervical compression: positive bilateral; Cervical distraction: positive bilateral for pain; Shoulder depression: positive bilateral; Bechterew's: negative; Ely's: positive bilateral; Kemp test: positive bilateral; Iliac compression: positive bilateral; Milgram's: positive bilateral; Patrick's Fabere: negative; Piriformis: negative; Straight leg raise: negative; Yeoman's: positive bilateral.

Spinal level: Subluxations found C1, C4, T1, T3, T4, T6, T8, T11, L1, L4, L5, left sacroiliac, right ilium.

Assessment

Response to care: patient tolerated today's treatment well & without incident.

Diagnosis: M99.03 (Segmental and somatic dysfunction of lumbar region), M54.51 (Vertebrogenic low back pain), M99.02 (Segmental and somatic dysfunction of thoracic region), M54.6 (Pain in thoracic spine), M99.01 (Segmental and somatic dysfunction of cervical region), M54.2 (Cervicalgia), G44.319 (Acute post-traumatic headache, not intractable), F07.81 (Postconcussional syndrome), M79.2 (Neuralgia and neuritis, unspecified), M79.18 (Myalgia, other site).

Diagnostic Procedures & Treatment Rendered

Region & treatment rationale: acute care region(s) -> (bilateral) cervical, thoracic, lumbar, pelvis, sacroiliac; rationale -> reduce pain intensity, reduce pain frequency, reduce tenderness, reduce tightness, improve functional capacity, improve mobility.

Subluxations found on assessment and adjusted: C1 right; C4 left; T1, T3, T4 post; T6, T8, T11, L1 ant; L4 left; L5 right; left sacroiliac; right ilium; Unless otherwise specified, all areas adjusted using: (diversified).

Physical Modalities: bilateral cervical, thoracic, lumbar, pelvis, sacroiliac intersegmental traction, myofascial release, therapeutic stretching (15 minutes).

Patient given: home exercise program: heat, ice and rest instructions.

Informed consent: Benefits/risks/alternatives discussed with patient, Patient had no questions, Patient agreed to begin care.

Plan of Care & Goals

Short term goals: start on 07/16/2024, bilateral **full spine** by 8 weeks -> decrease pain, decreased tenderness, improved ADL (100%).

Treatment schedule: 2x/week for 2 week(s) ; 1x/week for 6 week(s) ; 2x/month for 4 week(s) .

End of note. Electronically signed by Michael Fillat D.C on 07/17/2024 at 8:39am

Addendum: Upon reviewing the notes, noticed this note is dated 07/17/24 but first visit was 07/16/24. Possible finished the note the next morning. Or, a mis-date.

End of Addendum. Electronically signed by Michael Fillat D.C on 11/18/2024 at 10:29am

Thursday July 18, 2024 Provider: Michael Fillat D.C

Date of service: 07/18/2024

NPI#: 1801824404

Electronically signed by Michael Fillat D.C on 07/18/2024 at 10:55am

Subjective

Felt some relief for a few hours after adjustment, but had a rough day yesterday. Had a bad migraine and was dizzy so she could not get out of bed until 11:30.

Complaints: **primary** -> bilateral **low back**, acute onset immediately at time of injury, pain described as: achy, dull, sharp, tender, intensity: moderate, level: 5/10, range: 5-7; **secondary** -> bilateral **c/t pain and headaches**, acute onset immediately at time of injury, pain described as: achy, dull, sharp, stiff, tender, intensity: moderate, level: 5/10, range: 5-7.

Objective

+percussion c2-4 and L2-5. -valsavas.

Visual inspection of the patient revealed: -minors sign, no visible pain.

Mental status: Patient appeared: alert, awake, oriented to person, place, time and situation, able to show knowledge of current events, mildly confused.

Sensation: upper & lower extremity dermatomes: hypoesthesia light touch pin prick -> mostly c5-6 and L4-5.

Myofascial pain/involvement: left & right suboccipital: tightness, tenderness; left & right cervical paraspinal: tightness, tenderness; left & right thoracic paraspinal: tightness, tenderness; left & right thoracolumbar: tightness, tenderness; left & right lumbar paraspinal: tightness, tenderness.

Spinal tenderness on palpation: C2; C3; C4; L2; L3; L4; L5; left SI; right SI.

Spinal level: Subluxations found C1, C4, T1, T3, T4, T6, T8, T10, L1, L4, L5, left sacroiliac, right ilium.

Assessment

Response to care: patient tolerated today's treatment well & without incident.

Diagnosis: M99.03 (Segmental and somatic dysfunction of lumbar region), M54.51 (Vertebrogenic low back pain), M99.02 (Segmental and somatic dysfunction of thoracic region), M54.6 (Pain in thoracic spine), M99.01 (Segmental and somatic dysfunction of cervical region), M54.2 (Cervicalgia), G44.319 (Acute post-traumatic headache, not intractable), F07.81 (Postconcussional syndrome), M79.2 (Neuralgia and neuritis, unspecified), M79.18 (Myalgia, other site).

Diagnostic Procedures & Treatment Rendered

Region & treatment rationale: acute care region(s) -> (bilateral) cervical, thoracic, lumbar, pelvis, sacroiliac; rationale -> reduce pain intensity, reduce pain frequency, reduce tenderness, reduce tightness, improve functional capacity, improve mobility.

Subluxations found on assessment and adjusted: C1 right; C4 left; T1, T3, T4 post; T6, T8, T10, L1 ant; L4 left; L5 right; left sacroiliac; right ilium; Unless otherwise specified, all areas adjusted using: (diversified).

Physical Modalities: bilateral cervical, thoracic, lumbar, pelvis, sacroiliac intersegmental traction, myofascial release, therapeutic stretching (15 minutes).

Plan of Care & Goals

Plan: Adjusted a little easier today with less guarding/spasm during adjustment.

Short term goals: start on 07/16/2024, bilateral **full spine** by 8 weeks -> decrease pain, decreased tenderness, improved ADL (100%).

Treatment schedule: 2x/week for 1 week(s) ; 1x/week for 6 week(s) ; 2x/month for 4 week(s) .

End of note. Electronically signed by Michael Fillat D.C on 07/18/2024 at 10:55am

Monday July 22, 2024 Provider: Michael Fillat D.C

Date of service: 07/22/2024

NPI#: 1801824404

Electronically signed by Michael Fillat D.C on 07/22/2024 at 9:56am

Subjective

Adjustments definitely help and take the edge off. Did have a bad migraine on Saturday and had to take meds. LB is maybe a little better after last adjustment but it feels worse as day goes on.

Complaints: **primary** -> bilateral **low back**, acute onset immediately at time of injury, pain described as: achy, dull, sharp, tender, intensity: moderate, level: 5/10, range: 5-7; **secondary** -> bilateral **c/t pain and headaches**, acute onset immediately at time of injury, pain described as: achy, dull, sharp, stiff, tender, intensity: moderate, level: 6/10, range: 5-7.

Objective

+percussion c2-4 and L2-5.

Visual inspection of the patient revealed: -minors sign, no visible pain.

Mental status: Patient appeared: alert, awake, oriented to person, place, time and situation, able to show knowledge of current events, mildly confused.

Sensation: upper & lower extremity dermatomes: hypoesthesia light touch pin prick -> mostly c5-6 and L4-5.

Myofascial pain/involvement: left & right suboccipital: tightness, tenderness; left & right cervical paraspinal: tightness, tenderness; left & right thoracic paraspinal: tightness, tenderness; left & right thoracolumbar: tightness, tenderness; left & right lumbar paraspinal: tightness, tenderness.

Spinal tenderness on palpation: C2; C3; L2; L3; L4; L5; left SI; right SI.

Spinal level: Subluxations found C1, C5, T1, T3, T4, T6, T8, T10, L1, L4, L5, left sacroiliac, right ilium.

Assessment

Response to care: patient tolerated today's treatment well & without incident.

Diagnosis: M99.03 (Segmental and somatic dysfunction of lumbar region), M54.51 (Vertebrogenic low back pain), M99.02 (Segmental and somatic dysfunction of thoracic region), M54.6 (Pain in thoracic spine), M99.01 (Segmental and somatic dysfunction of cervical region), M54.2 (Cervicalgia), G44.319 (Acute post-traumatic headache, not intractable), F07.81 (Postconcussional syndrome), M79.2 (Neuralgia and neuritis, unspecified), M79.18 (Myalgia, other site).

Diagnostic Procedures & Treatment Rendered

Region & treatment rationale: acute care region(s) -> (bilateral) cervical, thoracic, lumbar, pelvis, sacroiliac; rationale -> reduce pain intensity, reduce pain frequency, reduce tenderness, reduce tightness, improve functional capacity, improve mobility.

Subluxations found on assessment and adjusted: C1 right; C5 left; T1, T3, T4 post; T6, T8, T10, L1 ant; L4 left; L5 right; left sacroiliac; right ilium; Unless otherwise specified, all areas adjusted using: (diversified).

Physical Modalities: bilateral cervical, thoracic, lumbar, pelvis, sacroiliac intersegmental traction, myofascial release, therapeutic stretching (15 minutes).

Plan of Care & Goals

Plan: A little more pain and difficulty with adjustment today then last but not as bad as first.

Short term goals: start on 07/16/2024, bilateral **full spine** by 8 weeks -> decrease pain, decreased tenderness, improved ADL (100%).

Treatment schedule: 2x/week for 1 week(s) see 3 if nec. ; 1x/week for 6 week(s) ; 2x/month for 4 week(s) .

End of note. Electronically signed by Michael Fillat D.C on 07/22/2024 at 9:56am

Wednesday July 24, 2024 Provider: Michael Fillat D.C

Date of service: 07/24/2024

NPI#: 1801824404

Electronically signed by Michael Fillat D.C on 07/24/2024 at 10:43am

Subjective

Did about the same since Monday. Still having pain in the same areas. More in the neck then elsewhere. Maybe not quite as intense but still as frequent. Woke today with a headache.

Complaints: **primary** -> bilateral **low back**, acute onset immediately at time of injury, pain described as: achy, dull, sharp, tender, intensity: moderate, level: 5/10, range: 5-7; **secondary** -> bilateral **c/t pain and headaches**, acute onset immediately at time of injury, pain described as: achy, dull, sharp, stiff, tender, intensity: moderate, level: 5/10, range: 5-7.

Objective

Visual inspection of the patient revealed: -minors sign, no visible pain.

Mental status: Patient appeared: alert, awake, oriented to person, place, time and situation, able to show knowledge of current events, mildly confused.

Myofascial pain/involvement: left & right suboccipital: tightness, tenderness; left & right cervical paraspinal: tightness, tenderness; left & right thoracic paraspinal: tightness, tenderness; left & right thoracolumbar: tightness, tenderness; left & right lumbar paraspinal: tightness, tenderness.

Spinal tenderness on palpation: C3; L3; L4; L5; left SI; right SI.

Spinal level: Subluxations found C1, C5, T1, T3, T6, T8, T10, L1, L4, L5, left sacroiliac, right ilium.

Assessment

Response to care: patient tolerated today's treatment well & without incident.

Diagnosis: M99.03 (Segmental and somatic dysfunction of lumbar region), M54.51 (Vertebrogenic low back pain), M99.02 (Segmental and somatic dysfunction of thoracic region), M54.6 (Pain in thoracic spine), M99.01 (Segmental and somatic dysfunction of cervical region), M54.2 (Cervicalgia), G44.319 (Acute post-traumatic headache, not intractable), F07.81 (Postconcussional syndrome), M79.2 (Neuralgia and neuritis, unspecified), M79.18 (Myalgia, other site).

Diagnostic Procedures & Treatment Rendered

Region & treatment rationale: acute care region(s) -> (bilateral) cervical, thoracic, lumbar, pelvis, sacroiliac; rationale -> reduce pain intensity, reduce pain frequency, reduce tenderness, reduce tightness, improve functional capacity, improve mobility.

Subluxations found on assessment and adjusted: C1 right; C5 left; T1, T3 post; T6, T8, T10, L1 ant; L4 left; L5 right; left sacroiliac; right ilium; Unless otherwise specified, all areas adjusted using: (diversified).

Physical Modalities: bilateral cervical, thoracic, lumbar, pelvis, sacroiliac intersegmental traction, myofascial release, therapeutic stretching (15 minutes).

Plan of Care & Goals

Short term goals: start on 07/16/2024, bilateral **full spine** by 8 weeks -> decrease pain, decreased tenderness, improved ADL (100%).

Treatment schedule: 2x/week for 1 week(s) see 3 if nec. ; 1x/week for 6 week(s) ; 2x/month for 4 week(s) .

End of note. Electronically signed by Michael Fillat D.C on 07/24/2024 at 10:43am

Friday July 26, 2024 Provider: Michael Fillat D.C

Date of service: 07/26/2024

NPI#: 1801824404

Electronically signed by Michael Fillat D.C on 07/26/2024 at 11:15am

Subjective

Returned because she did not feel like she could go through weekend without an adjustment. Did feel relief day of last treatment but about the same today and had a couple headaches since last. Her LB popped this morning and a little pain with that but not too bad. She was worried it might make it feel worse but doesn't seem to have.

Complaints: **primary** -> bilateral **low back**, acute onset immediately at time of injury, pain described as: achy, dull, sharp, tender, intensity: moderate, level: 5/10, range: 5-7; **secondary** -> bilateral **c/t pain and headaches**, acute onset immediately at time of injury, pain described as: achy, dull, sharp, stiff, tender, intensity: moderate, level: 5/10, range: 5-7.

Objective

Visual inspection of the patient revealed: -minors sign, no visible pain.

Mental status: Patient appeared: alert, awake, oriented to person, place, time and situation, able to show knowledge of current events, mildly confused.

Myofascial pain/involvement: left & right suboccipital: tightness, tenderness; left & right cervical paraspinal: tightness, tenderness; left & right thoracic paraspinal: tightness, tenderness; left & right thoracolumbar: tightness, tenderness; left & right lumbar paraspinal: tightness, tenderness.

Spinal tenderness on palpation: C3; L4; L5; left SI; right SI.

Spinal level: Subluxations found C1, C6, T1, T3, T6, T8, T10, L1, L3, L5, left sacroiliac, right ilium.

Assessment

Response to care: patient tolerated today's treatment well & without incident.

Diagnosis: M99.03 (Segmental and somatic dysfunction of lumbar region), M54.51 (Vertebrogenic low back pain), M99.02 (Segmental and somatic dysfunction of thoracic region), M54.6 (Pain in thoracic spine), M99.01 (Segmental and somatic dysfunction of cervical region), M54.2 (Cervicalgia), G44.319 (Acute post-traumatic headache, not intractable), F07.81 (Postconcussional syndrome), M79.2 (Neuralgia and neuritis, unspecified), M79.18 (Myalgia, other site).

Diagnostic Procedures & Treatment Rendered

Region & treatment rationale: acute care region(s) -> (bilateral) cervical, thoracic, lumbar, pelvis, sacroiliac; rationale -> reduce pain intensity, reduce pain frequency, reduce tenderness, reduce tightness, improve functional capacity, improve mobility.

Subluxations found on assessment and adjusted: C1 right; C6 left; T1, T3 post; T6, T8, T10, L1 ant; L3 left; L5

right; left sacroiliac; right ilium; Unless otherwise specified, all areas adjusted using: (diversified).

Physical Modalities: bilateral cervical, thoracic, lumbar, pelvis, sacroiliac intersegmental traction, myofascial release, therapeutic stretching (15 minutes).

Plan of Care & Goals

Short term goals: start on 07/16/2024, bilateral **full spine** by 8 weeks -> decrease pain, decreased tenderness, improved ADL (100%).

Treatment schedule: 2x/week for 1 week(s) ; 1x/week for 5 week(s) ; 2x/month for 4 week(s) .

End of note. Electronically signed by Michael Fillat D.C on 07/26/2024 at 11:15am

Tuesday July 30, 2024 Provider: Michael Fillat D.C

Date of service: 07/30/2024

NPI#: 1801824404

Electronically signed by Michael Fillat D.C on 07/30/2024 at 10:14am

Subjective

Did a little better after last, but still having frequent symptoms. Mostly neck and headache stuff. LB is doing better but she has LBP today. She thinks she might have slept wrong and that caused a headache today. She only had one minor headache since last until today. She also said she has some difficulty driving on the freeway and it triggers migraines. Today she took side roads to get here.

Complaints: **primary** -> bilateral **low back**, acute onset immediately at time of injury, pain described as: achy, dull, sore, tender, intensity: mild to moderate, level: 4/10; **secondary** -> bilateral **c/t pain and headaches**, acute onset immediately at time of injury, pain described as: achy, dull, sharp, stiff, tender, intensity: moderate, level: 5/10.

Objective

Visual inspection of the patient revealed: -minors sign, no visible pain.

+Rhombergs test and did vision field test. To the upper right she had nystagmus. We used the laser on the brain to decrease the sway and the nystagmus. Will do for a few treatments to work towards improvement and correction. Possibly will help with the headaches.

Mental status: Patient appeared: alert, awake, oriented to person, place, time and situation, able to show knowledge of current events, mildly confused.

Myofascial pain/involvement: left & right suboccipital: tightness, tenderness; left & right cervical paraspinal: tightness, tenderness; left & right thoracic paraspinal: tightness, tenderness; left & right thoracolumbar: tightness, tenderness; left & right lumbar paraspinal: tightness.

Spinal tenderness on palpation: C3; L4; L5.

Spinal level: Subluxations found C1, C4, T1, T3, T6, T10, L3, L5, left sacroiliac, right ilium.

Assessment

Response to care: patient tolerated today's treatment well & without incident.

Diagnosis: M99.03 (Segmental and somatic dysfunction of lumbar region), M54.51 (Vertebrogenic low back pain), M99.02 (Segmental and somatic dysfunction of thoracic region), M54.6 (Pain in thoracic spine), M99.01 (Segmental and somatic dysfunction of cervical region), M54.2 (Cervicalgia), G44.319 (Acute post-traumatic headache, not intractable), F07.81 (Postconcussional syndrome), M79.2 (Neuralgia and neuritis, unspecified), M79.18 (Myalgia, other site).

Diagnostic Procedures & Treatment Rendered

Region & treatment rationale: acute care region(s) -> (bilateral) cervical, thoracic, lumbar, pelvis, sacroiliac; rationale -> reduce pain intensity, reduce pain frequency, reduce tenderness, reduce tightness, improve functional capacity, improve mobility.

Subluxations found on assessment and adjusted: C1 right; C4 left; T1, T3 post; T10 ant; L3 left; L5 right; left sacroiliac; right ilium; Unless otherwise specified, all areas adjusted using: (diversified).

Physical Modalities: bilateral cervical, thoracic, lumbar, pelvis, sacroiliac intersegmental traction, myofascial release, therapeutic stretching (15 minutes).

Plan of Care & Goals

Short term goals: start on 07/16/2024, bilateral **full spine** by 8 weeks -> decrease pain, decreased tenderness, improved ADL (100%).

Treatment schedule: 2x/week for 1 week(s) ; 1x/week for 5 week(s) ; 2x/month for 4 week(s) .

End of note. Electronically signed by Michael Fillat D.C on 07/30/2024 at 10:14am

Friday August 2, 2024 Provider: Michael Fillat D.C

Date of service: 08/02/2024

NPI#: 1801824404

Electronically signed by Michael Fillat D.C on 08/02/2024 at 10:24am

Subjective

She felt like the laser treatment helped. It helped to decrease some of her anxiety while driving. Also no migraines caused by driving. Had one headache since last, but not a migraine and did not affect her ADLs like they usually do. Back and neck pain about the same.

Complaints: **primary** -> bilateral **low back**, acute onset immediately at time of injury, pain described as: achy, dull, sore, tender, intensity: mild to moderate, level: 4/10; **secondary** -> bilateral **c/t pain and headaches**, acute onset immediately at time of injury, pain described as: achy, dull, sharp, stiff, tender, intensity: moderate, level: 5/10.

Objective

Visual inspection of the patient revealed: -minors sign, no visible pain.

+Rhomberts test and did vision field test. To the upper right and upper left she had nystagmus. We used the laser on the brain to decrease the sway and the nystagmus.

Mental status: Patient appeared: alert, awake, oriented to person, place, time and situation, able to show knowledge of current events, mildly confused.

Myofascial pain/involvement: left & right suboccipital: tightness, tenderness; left & right cervical paraspinal: tightness, tenderness; left & right thoracic paraspinal: tightness, tenderness; left & right thoracolumbar: tightness, tenderness; left & right lumbar paraspinal: tightness.

Spinal tenderness on palpation: C3; L5.

Spinal level: Subluxations found C1, C3, T1, T4, T6, T11, L3, L5, left sacroiliac, right ilium.

Assessment

Response to care: patient tolerated today's treatment well & without incident.

Diagnosis: M99.03 (Segmental and somatic dysfunction of lumbar region), M54.51 (Vertebrogenic low back pain), M99.02 (Segmental and somatic dysfunction of thoracic region), M54.6 (Pain in thoracic spine), M99.01 (Segmental and somatic dysfunction of cervical region), M54.2 (Cervicalgia), G44.319 (Acute post-traumatic headache, not intractable), F07.81 (Postconcussional syndrome), M79.2 (Neuralgia and neuritis, unspecified), M79.18 (Myalgia, other site).

Diagnostic Procedures & Treatment Rendered

Region & treatment rationale: acute care region(s) -> (bilateral) cervical, thoracic, lumbar, pelvis, sacroiliac; rationale -> reduce pain intensity, reduce pain frequency, reduce tenderness, reduce tightness, improve functional capacity, improve mobility.

Subluxations found on assessment and adjusted: C1 right; C3 left; T1, T4 post; T11 ant; L3 left; L5 right; left sacroiliac; right ilium; Unless otherwise specified, all areas adjusted using: (diversified).

Physical Modalities: bilateral cervical, thoracic, lumbar, pelvis, sacroiliac intersegmental traction, myofascial release, therapeutic stretching (15 minutes).

Plan of Care & Goals

Short term goals: start on 07/16/2024, bilateral **full spine** by 8 weeks -> decrease pain, decreased tenderness, improved ADL (100%).

Treatment schedule: 2x/week for 2 week(s) ; 1x/week for 3 week(s) ; 2x/month for 4 week(s) .

End of note. Electronically signed by Michael Fillat D.C on 08/02/2024 at 10:24am

Tuesday August 6, 2024 Provider: Michael Fillat D.C

Date of service: 08/06/2024

NPI#: 1801824404

Electronically signed by Michael Fillat D.C on 08/06/2024 at 11:01am

Subjective

Really feels like the laser is helping. Especially with anxious driving but pain also. No sharp pain since last. Still headaches but also not as bad as usual.

Complaints: **primary** -> bilateral **low back**, acute onset immediately at time of injury, pain described as: achy, dull, sore, tender, intensity: mild, level: 3/10; **secondary** -> bilateral **c/t pain and headaches**, acute onset immediately at time of injury, pain described as: achy, dull, stiff, tender, intensity: mild to moderate, level: 4/10.

Objective

Visual inspection of the patient revealed: -minors sign, no visible pain.

+Rhomberts test and did vision field test. To the upper right and upper left she had nystagmus but less then last time. We used the laser on the brain to decrease the sway and the nystagmus.

Mental status: Patient appeared: alert, awake, oriented to person, place, time and situation, able to to show knowledge of current events, mildly confused.

Myofascial pain/involvement: left & right suboccipital: tightness, tenderness; left & right cervical paraspinal: tightness, tenderness; left & right thoracic paraspinal: tightness, tenderness; left & right thoracolumbar: tightness, tenderness; left & right lumbar paraspinal: tightness.

Spinal tenderness on palpation: C3; L5.

Spinal level: Subluxations found C1, C3, T1, T3, T6, T11, L3, L5, left sacroiliac.

Assessment

Response to care: patient tolerated today's treatment well & without incident.

Diagnosis: M99.03 (Segmental and somatic dysfunction of lumbar region), M54.51 (Vertebrogenic low back pain), M99.02 (Segmental and somatic dysfunction of thoracic region), M54.6 (Pain in thoracic spine), M99.01 (Segmental and somatic dysfunction of cervical region), M54.2 (Cervicalgia), G44.319 (Acute post-traumatic headache, not intractable), F07.81 (Postconcussional syndrome), M79.2 (Neuralgia and neuritis, unspecified), M79.18 (Myalgia, other site).

Diagnostic Procedures & Treatment Rendered

Region & treatment rationale: acute care region(s) -> (bilateral) cervical, thoracic, lumbar, pelvis, sacroiliac; rationale -> reduce pain intensity, reduce pain frequency, reduce tenderness, reduce tightness, improve functional capacity, improve mobility.

Subluxations found on assessment and adjusted: C1 right; C3 left; T1, T3, T6 post; T11 ant; L3 left; L5 right; left sacroiliac; Unless otherwise specified, all areas adjusted using: (diversified).

Physical Modalities: bilateral cervical, thoracic, lumbar, pelvis, sacroiliac cold laser, intersegmental traction, myofascial release, therapeutic stretching (25 minutes).

Plan of Care & Goals

Short term goals: start on 07/16/2024, bilateral **full spine** by 8 weeks -> decrease pain, decreased tenderness, improved ADL (100%).

Treatment schedule: 2x/week for 2 week(s) ; 1x/week for 3 week(s) ; 2x/month for 4 week(s) .

End of note. Electronically signed by Michael Fillat D.C on 08/06/2024 at 11:01am

Friday August 9, 2024 Provider: Michael Fillat D.C

Date of service: 08/09/2024

NPI#: 1801824404

Electronically signed by Michael Fillat D.C on 08/09/2024 at 10:05am

Subjective

About the same as last. Had some stress and that caused a bit of a flare up. One headache that was worse then they have been for the past couple weeks. Feels like LB is better then c/t now though.

Complaints: **primary** -> bilateral **low back**, acute onset immediately at time of injury, pain described as: dull, sore, tender, intensity: mild, level: 3/10; **secondary** -> bilateral **c/t pain and headaches**, acute onset immediately at time of injury, pain described as: achy, dull, stiff, tender, intensity: moderate, level: 5/10.

Objective

Visual inspection of the patient revealed: -minors sign, no visible pain.

+Rhomberts test and did vision field test. A little more nystagmus on fields of gaze then last time. We used the laser on the brain to decrease the sway and the nystagmus. Able to make a correction.

Mental status: Patient appeared: alert, awake, oriented to person, place, time and situation, able to show knowledge of current events, mildly confused.

Myofascial pain/involvement: left & right suboccipital: tightness, tenderness; left & right cervical paraspinal: tightness, tenderness; left & right thoracic paraspinal: tightness, tenderness; left & right thoracolumbar: tightness, tenderness; left & right lumbar paraspinal: tightness, tenderness.

Spinal tenderness on palpation: none.

Spinal level: Subluxations found C1, C4, T2, T4, T11, L3, L5, left sacroiliac.

Assessment

Response to care: patient tolerated today's treatment well & without incident.

Diagnosis: M99.03 (Segmental and somatic dysfunction of lumbar region), M54.51 (Vertebrogenic low back pain), M99.02 (Segmental and somatic dysfunction of thoracic region), M54.6 (Pain in thoracic spine), M99.01 (Segmental and somatic dysfunction of cervical region), M54.2 (Cervicalgia), G44.319 (Acute post-traumatic headache, not intractable), F07.81 (Postconcussional syndrome), M79.2 (Neuralgia and neuritis, unspecified), M79.18 (Myalgia, other site).

Diagnostic Procedures & Treatment Rendered

Region & treatment rationale: acute care region(s) -> (bilateral) cervical, thoracic, lumbar, pelvis, sacroiliac; rationale -> reduce pain intensity, reduce pain frequency, reduce tenderness, reduce tightness, improve functional capacity, improve mobility.

Michael T. Fillat, D.C. 10444 Clairemont Mesa Blvd San Diego, CA 92124-1320

Encounter dated 08/09/2024 for Abbie Gladstone #8774

DOB:12/14/1999 Today's date: 11/18/2024

Subluxations found on assessment and adjusted: C1 right; C4 left; T2, T4 post; T11 ant; L3 left; L5 right; left sacroiliac; Unless otherwise specified, all areas adjusted using: (diversified).

Physical Modalities: bilateral cervical, thoracic, lumbar, pelvis, sacroiliac cold laser, intersegmental traction, myofascial release, therapeutic stretching (25 minutes).

Plan of Care & Goals

Short term goals: start on 07/16/2024, bilateral **full spine** by 8 weeks -> decrease pain, decreased tenderness, improved ADL (100%).

Treatment schedule: 2x/week for 1 week(s) ; 1x/week for 3 week(s) ; 2x/month for 4 week(s) .

End of note. Electronically signed by Michael Fillat D.C on 08/09/2024 at 10:05am

Tuesday August 13, 2024 Provider: Michael Fillat D.C

Date of service: 08/13/2024

NPI#: 1801824404

Electronically signed by Michael Fillat D.C on 08/13/2024 at 10:59am

Subjective

Felt some relief from last and no headaches until this morning. Had a rough day at work sitting all day yesterday which brought on c/t pain and then she didn't sleep well as a result and woke this morning with a headache and still has it. Is feeling less anxiety with driving and feels more balanced with the laser.

Complaints: **primary** -> bilateral **low back**, acute onset immediately at time of injury, pain radiation: no radiation, pain described as: dull, sore, tender, intensity: mild, level: 2/10; **secondary** -> bilateral **c/t pain and headaches**, acute onset immediately at time of injury, pain radiation: no radiation, pain described as: achy, dull, stiff, tender, intensity: moderate, level: 5/10.

Objective

Visual inspection of the patient revealed: -minors sign, no visible pain.

-Rhomberts test and did vision field test. No nystagmus.

Mental status: Patient appeared: alert, awake, oriented to person, place, time and situation, able to to show knowledge of current events, mildly confused.

Myofascial pain/involvement: left & right suboccipital: tightness, tenderness; left & right cervical paraspinal: tightness, tenderness; left & right thoracic paraspinal: tightness, tenderness; left & right thoracolumbar: tightness; left & right lumbar paraspinal: tightness.

Spinal tenderness on palpation: none.

Spinal level: Subluxations found C1, C2, T2, T4, T11, L4, L5, left sacroiliac.

Assessment

Response to care: patient tolerated today's treatment well & without incident.

Diagnosis: M99.03 (Segmental and somatic dysfunction of lumbar region), M54.51 (Vertebrogenic low back pain), M99.02 (Segmental and somatic dysfunction of thoracic region), M54.6 (Pain in thoracic spine), M99.01 (Segmental and somatic dysfunction of cervical region), M54.2 (Cervicalgia), G44.319 (Acute post-traumatic headache, not intractable), F07.81 (Postconcussional syndrome), M79.2 (Neuralgia and neuritis, unspecified), M79.18 (Myalgia, other site).

Diagnostic Procedures & Treatment Rendered

Region & treatment rationale: acute care region(s) -> (bilateral) cervical, thoracic, lumbar, pelvis, sacroiliac; rationale -> reduce pain intensity, reduce pain frequency, reduce tenderness, reduce tightness, improve functional capacity, improve mobility.

Subluxations found on assessment and adjusted: C1 right; C2 left; T2, T4 post; T11 ant; L4 left; L5 right; left sacroiliac; Unless otherwise specified, all areas adjusted using: (diversified).

Physical Modalities: bilateral cervical, thoracic, lumbar, pelvis, sacroiliac intersegmental traction, myofascial release, therapeutic stretching (15 minutes).

Plan of Care & Goals

Plan: Decided not to do laser today to see how she does as the neurological symptoms were less to none and her anxiety being down.

Short term goals: start on 07/16/2024, bilateral **full spine** by 8 weeks -> decrease pain, decreased tenderness, improved ADL (100%).

Treatment schedule: 2x/week for 1 week(s) ; 1x/week for 3 week(s) ; 2x/month for 4 week(s) .

End of note. Electronically signed by Michael Fillat D.C on 08/13/2024 at 10:59am

Monday August 19, 2024 Provider: Michael Fillat D.C

Date of service: 08/19/2024

NPI#: 1801824404

Electronically signed by Michael Fillat D.C on 08/19/2024 at 2:45pm

Subjective

Did better after last and was having a good week with pain and driving. Then Friday she had some back pain and was going to come in but thought it was not enough to need an adjustment. Saturday decided to go on pretty easy bike ride around the lake and was pretty sore on Sunday and today. Feels like a headache might be coming on.

Complaints: **primary** -> bilateral **low back**, acute onset immediately at time of injury, pain radiation: no radiation, pain described as: dull, sore, tender, intensity: mild, level: 3/10; **secondary** -> bilateral **c/t pain and headaches**, acute onset immediately at time of injury, pain radiation: no radiation, pain described as: achy, dull, stiff, tender, intensity: mild to moderate, level: 4/10.

Objective

Visual inspection of the patient revealed: -minors sign, no visible pain.

+Rhomberts test and some nystagmus in vision field test. So, we will do laser again.

Mental status: Patient appeared: alert, awake, oriented to person, place, time and situation, able to show knowledge of current events, mildly confused.

Myofascial pain/involvement: left & right suboccipital: tightness, tenderness; left & right cervical paraspinal: tightness, tenderness; left & right thoracic paraspinal: tightness, tenderness; left & right thoracolumbar: tightness; left & right lumbar paraspinal: tightness.

Spinal tenderness on palpation: none.

Spinal level: Subluxations found C1, C2, T2, T5, T11, L4, L5, left sacroiliac.

Assessment

Response to care: patient tolerated today's treatment well & without incident.

Diagnosis: M99.03 (Segmental and somatic dysfunction of lumbar region), M54.51 (Vertebrogenic low back pain), M99.02 (Segmental and somatic dysfunction of thoracic region), M54.6 (Pain in thoracic spine), M99.01 (Segmental and somatic dysfunction of cervical region), M54.2 (Cervicalgia), G44.319 (Acute post-traumatic headache, not intractable), F07.81 (Postconcussional syndrome), M79.2 (Neuralgia and neuritis, unspecified), M79.18 (Myalgia, other site).

Diagnostic Procedures & Treatment Rendered

Region & treatment rationale: acute care region(s) -> (bilateral) cervical, thoracic, lumbar, pelvis, sacroiliac; rationale -> reduce pain intensity, reduce pain frequency, reduce tenderness, reduce tightness, improve functional capacity, improve mobility.

Subluxations found on assessment and adjusted: C1 right; C2 left; T2, T5 post; T11 ant; L4 left; L5 right; left sacroiliac; Unless otherwise specified, all areas adjusted using: (diversified).

Physical Modalities: bilateral cervical, thoracic, lumbar, pelvis, sacroiliac cold laser, intersegmental traction, myofascial release, therapeutic stretching (25 minutes).

Plan of Care & Goals

Short term goals: start on 07/16/2024, bilateral **full spine** by 8 weeks -> decrease pain, decreased tenderness, improved ADL (100%).

Treatment schedule: 1x/week for 3 week(s) Try to go a week; 2x/month for 4 week(s) .

End of note. Electronically signed by Michael Fillat D.C on 08/19/2024 at 2:45pm

Monday August 26, 2024 Provider: Michael Fillat D.C

Date of service: 08/26/2024

NPI#: 1801824404

Electronically signed by Michael Fillat D.C on 08/26/2024 at 4:28pm

Subjective

Feels like she is getting better and feeling less pain. Felt like laser really helped with anxiety of driving after last. Did not have to ask her Mom to drive her this past week. She is still having some days that are not as good as others, but not bad enough to feel like she has to get in here everytime. We talked about a few more adjustments and we will be finished and I told her to try going closer to ten days this time. She also went on another bike ride and was not as sore after.

Complaints: **primary** -> bilateral **low back**, acute onset immediately at time of injury, pain radiation: no radiation, pain described as: dull, sore, tender, intensity: mild, level: 3/10; **secondary** -> bilateral **c/t pain and headaches**, acute onset immediately at time of injury, pain radiation: no radiation, pain described as: achy, dull, stiff, tender, intensity: mild, level: 3/10.

Objective

Visual inspection of the patient revealed: -minors sign, no visible pain.

+Rhomberts test but not as bad, and some nystagmus in vision field test. So, we will do laser again.

Mental status: Patient appeared: alert, awake, oriented to person, place, time and situation, able to show knowledge of current events, mildly confused.

Myofascial pain/involvement: left & right suboccipital: tightness, tenderness; left & right cervical paraspinal: tightness, tenderness; left & right thoracic paraspinal: tightness, tenderness; left & right thoracolumbar: tightness; left & right lumbar paraspinal: tightness.

Spinal tenderness on palpation: none.

Spinal level: Subluxations found C1, C2, T2, T4, T11, L4, L5, left sacroiliac.

Assessment

Response to care: patient tolerated today's treatment well & without incident.

Diagnosis: M99.03 (Segmental and somatic dysfunction of lumbar region), M54.51 (Vertebrogenic low back pain), M99.02 (Segmental and somatic dysfunction of thoracic region), M54.6 (Pain in thoracic spine), M99.01 (Segmental and somatic dysfunction of cervical region), M54.2 (Cervicalgia), G44.319 (Acute post-traumatic headache, not intractable), F07.81 (Postconcussional syndrome), M79.2 (Neuralgia and neuritis, unspecified), M79.18 (Myalgia, other site).

Diagnostic Procedures & Treatment Rendered

Region & treatment rationale: acute care region(s) -> (bilateral) cervical, thoracic, lumbar, pelvis, sacroiliac; rationale -> reduce pain intensity, reduce pain frequency, reduce tenderness, reduce tightness, improve functional capacity, improve mobility.

Subluxations found on assessment and adjusted: C1 right; C2 left; T2, T4 post; T11 ant; L4 left; L5 right; left sacroiliac; Unless otherwise specified, all areas adjusted using: (diversified).

Physical Modalities: bilateral cervical, thoracic, lumbar, pelvis, sacroiliac cold laser, intersegmental traction, myofascial release, therapeutic stretching (25 minutes).

Plan of Care & Goals

Michael T. Fillat, D.C. 10444 Clairemont Mesa Blvd San Diego, CA 92124-1320

Encounter dated 08/26/2024 for Abbie Gladstone #8774

DOB:12/14/1999 Today's date: 11/18/2024

Short term goals: start on 07/16/2024, bilateral **full spine** by 8 weeks -> decrease pain, decreased tenderness, improved ADL (100%).

Treatment schedule: 1x/week for 2 week(s) Try to go a week; 2x/month for 4 week(s) .

End of note. Electronically signed by Michael Fillat D.C on 08/26/2024 at 4:28pm

Wednesday September 4, 2024 Provider: Michael Fillat D.C

Date of service: 09/04/2024

NPI#: 1801824404

Electronically signed by Michael Fillat D.C on 09/04/2024 at 3:51pm

Subjective

Did even better after last so she tried to go longer. Was able to make it until today but yesterday had more pain then the rest of the days and a migraine.

Complaints: **primary** -> bilateral **low back**, acute onset immediately at time of injury, pain radiation: no radiation, pain described as: dull, sore, tender, intensity: mild, level: 3/10; **secondary** -> bilateral **c/t pain and headaches**, acute onset immediately at time of injury, pain radiation: no radiation, pain described as: achy, dull, stiff, tender, intensity: mild, level: 3/10.

Objective

Visual inspection of the patient revealed: -minors sign, no visible pain.

+Rhomberts test but not as bad, Vision field test wnl. So, we will do laser again.

Mental status: Patient appeared: alert, awake, oriented to person, place, time and situation, able to show knowledge of current events, mildly confused.

Myofascial pain/involvement: left & right suboccipital: tightness, tenderness; left & right cervical paraspinal: tightness, tenderness; left & right thoracic paraspinal: tightness.

Spinal tenderness on palpation: none.

Spinal level: Subluxations found C1, C2, T2, T4, L5, left sacroiliac.

Assessment

Response to care: patient tolerated today's treatment well & without incident.

Diagnosis: M99.03 (Segmental and somatic dysfunction of lumbar region), M54.51 (Vertebrogenic low back pain), M99.02 (Segmental and somatic dysfunction of thoracic region), M54.6 (Pain in thoracic spine), M99.01 (Segmental and somatic dysfunction of cervical region), M54.2 (Cervicalgia), G44.319 (Acute post-traumatic headache, not intractable), F07.81 (Postconcussional syndrome), M79.2 (Neuralgia and neuritis, unspecified), M79.18 (Myalgia, other site).

Diagnostic Procedures & Treatment Rendered

Region & treatment rationale: acute care region(s) -> (bilateral) cervical, thoracic, lumbar, pelvis, sacroiliac;

rationale -> reduce pain intensity, reduce pain frequency, reduce tenderness, reduce tightness, improve functional capacity, improve mobility.

Subluxations found on assessment and adjusted: C1 right; C2 left; T2, T4 post; L5 right; left sacroiliac; Unless otherwise specified, all areas adjusted using: (diversified).

Physical Modalities: bilateral cervical, thoracic, lumbar, pelvis, sacroiliac cold laser, intersegmental traction, myofascial release, therapeutic stretching (25 minutes).

Plan of Care & Goals

Short term goals: start on 07/16/2024, bilateral **full spine** by 8 weeks -> decrease pain, decreased tenderness, improved ADL (100%).

Treatment schedule: 1x/week for 2 week(s) ; 2x/month for 2 week(s) .

End of note. Electronically signed by Michael Fillat D.C on 09/04/2024 at 3:51pm

Friday September 13, 2024 Provider: Michael Fillat D.C

Date of service: 09/13/2024

NPI#: 1801824404

Electronically signed by Michael Fillat D.C on 09/13/2024 at 1:16pm

Subjective

Did pretty well again until the last 3 days. Had more pain and a headache, but not migraine last 3 days. Getting back to yoga again. Had not done it since accident so did it today for first time and no worse. Also going to try a flat bike ride over weekend.

Complaints: **primary** -> bilateral **low back**, acute onset immediately at time of injury, pain radiation: no radiation, pain described as: dull, sore, tender, intensity: mild, level: 3/10; **secondary** -> bilateral **c/t pain and headaches**, acute onset immediately at time of injury, pain radiation: no radiation, pain described as: achy, dull, stiff, tender, intensity: mild, level: 3/10.

Objective

Visual inspection of the patient revealed: -minors sign, no visible pain.

-Rhombberg's test.

Mental status: Patient appeared: alert, awake, oriented to person, place, time and situation, able to show knowledge of current events, mildly confused.

Myofascial pain/involvement: left & right suboccipital: tightness, tenderness; left & right cervical paraspinal: tightness, tenderness; left & right thoracic paraspinal: tightness.

Spinal tenderness on palpation: none.

Spinal level: Subluxations found C1, C2, T4, left sacroiliac.

Assessment

Response to care: patient tolerated today's treatment well & without incident.

Diagnosis: M99.03 (Segmental and somatic dysfunction of lumbar region), M54.51 (Vertebrogenic low back pain), M99.02 (Segmental and somatic dysfunction of thoracic region), M54.6 (Pain in thoracic spine), M99.01 (Segmental and somatic dysfunction of cervical region), M54.2 (Cervicalgia), G44.319 (Acute post-traumatic headache, not intractable), F07.81 (Postconcussional syndrome), M79.2 (Neuralgia and neuritis, unspecified), M79.18 (Myalgia, other site).

Diagnostic Procedures & Treatment Rendered

Region & treatment rationale: acute care region(s) -> (bilateral) cervical, thoracic, lumbar, pelvis, sacroiliac; rationale -> reduce pain intensity, reduce pain frequency, reduce tenderness, reduce tightness, improve functional capacity, improve mobility.

Subluxations found on assessment and adjusted: C1 right; C2 left; T4 post; left sacroiliac; Unless otherwise specified, all areas adjusted using: (diversified).

Physical Modalities: bilateral cervical, thoracic, lumbar, pelvis, sacroiliac intersegmental traction, myofascial release, therapeutic stretching (15 minutes).

Plan of Care & Goals

Short term goals: start on 07/16/2024, bilateral **full spine** by 8 weeks -> decrease pain, decreased tenderness, improved ADL (100%).

Treatment schedule: 1x/week for 1 week(s) ; 2x/month for 2 week(s) .

End of note. Electronically signed by Michael Fillat D.C on 09/13/2024 at 1:16pm

Friday September 20, 2024 Provider: Michael Fillat D.C

Date of service: 09/20/2024

NPI#: 1801824404

Electronically signed by Michael Fillat D.C on 09/20/2024 at 12:35pm

Subjective

Feeling out of sorts all week. Maybe overdid it last weekend because she was feeling better. Had a migraine on Sunday and has had a headache last 3 days. Tired.

Complaints: **primary** -> bilateral **low back**, acute onset immediately at time of injury, pain radiation: no radiation, pain described as: dull, sore, tender, intensity: mild, level: 3/10; **secondary** -> bilateral **c/t pain and headaches**, acute onset immediately at time of injury, pain radiation: no radiation, pain described as: achy, dull, stiff, tender, intensity: mild, level: 3/10.

Objective

Visual inspection of the patient revealed: -minors sign, no visible pain.

Mental status: Patient appeared: alert, awake, oriented to person, place, time and situation, able to show knowledge of current events, mildly confused.

Myofascial pain/involvement: left & right suboccipital: tightness, tenderness; left & right cervical paraspinal: tightness, tenderness; left & right thoracic paraspinal: tightness.

Spinal tenderness on palpation: none.

Spinal level: Subluxations found C1, C2, T2, T4.

Assessment

Response to care: patient tolerated today's treatment well & without incident.

Diagnosis: M99.03 (Segmental and somatic dysfunction of lumbar region), M54.51 (Vertebrogenic low back pain), M99.02 (Segmental and somatic dysfunction of thoracic region), M54.6 (Pain in thoracic spine), M99.01 (Segmental and somatic dysfunction of cervical region), M54.2 (Cervicalgia), G44.319 (Acute post-traumatic headache, not intractable), F07.81 (Postconcussional syndrome), M79.2 (Neuralgia and neuritis, unspecified), M79.18 (Myalgia, other site).

Diagnostic Procedures & Treatment Rendered

Region & treatment rationale: acute care region(s) -> (bilateral) cervical, thoracic, lumbar, pelvis, sacroiliac; rationale -> reduce pain intensity, reduce pain frequency, reduce tenderness, reduce tightness, improve functional capacity, improve mobility.

Subluxations found on assessment and adjusted: C1 right; C2 left; T2, T4 post; Unless otherwise specified, all areas adjusted using: (diversified).

Physical Modalities: bilateral cervical, thoracic, lumbar, pelvis, sacroiliac intersegmental traction, myofascial release, therapeutic stretching (15 minutes).

Plan of Care & Goals

Short term goals: start on 07/16/2024, bilateral **full spine** by 8 weeks -> decrease pain, decreased tenderness, improved ADL (100%).

Treatment schedule: 1x/week for 2 week(s) stay on schedule of once a week. Call if needs to get in sooner based on today's adjustment.

End of note. Electronically signed by Michael Fillat D.C on 09/20/2024 at 12:35pm

Friday September 27, 2024 Provider: Michael Fillat D.C

Date of service: 09/27/2024

NPI#: 1801824404

Electronically signed by Michael Fillat D.C on 09/27/2024 at 11:29am

Subjective

LB doing better. C/s and C/t still pain since last but not as frequent. Felt like she made it a week in better shape

and easier then after most other treatments.

Complaints: **primary** -> bilateral **low back**, acute onset immediately at time of injury, pain radiation: no radiation, pain described as: stiff, intensity: mild, level: 1/10; **secondary** -> bilateral **c/t pain and headaches**, acute onset immediately at time of injury, pain radiation: no radiation, pain described as: achy, dull, stiff, tender, intensity: mild to moderate, level: 4/10.

Objective

Visual inspection of the patient revealed: -minors sign, no visible pain.

Mental status: Patient appeared: alert, awake, oriented to person, place, time and situation, able to show knowledge of current events, mildly confused.

Myofascial pain/involvement: left & right suboccipital: tightness, tenderness; left & right cervical paraspinal: tightness, tenderness; left & right thoracic paraspinal: tightness.

Spinal tenderness on palpation: none.

Spinal level: Subluxations found C1, C3, T1, T2, T4, T6.

Assessment

Response to care: patient tolerated today's treatment well & without incident.

Diagnosis: M99.03 (Segmental and somatic dysfunction of lumbar region), M54.51 (Vertebrogenic low back pain), M99.02 (Segmental and somatic dysfunction of thoracic region), M54.6 (Pain in thoracic spine), M99.01 (Segmental and somatic dysfunction of cervical region), M54.2 (Cervicalgia), G44.319 (Acute post-traumatic headache, not intractable), F07.81 (Postconcussional syndrome), M79.2 (Neuralgia and neuritis, unspecified), M79.18 (Myalgia, other site).

Diagnostic Procedures & Treatment Rendered

Region & treatment rationale: acute care region(s) -> (bilateral) cervical, thoracic, lumbar, pelvis, sacroiliac; rationale -> reduce pain intensity, reduce pain frequency, reduce tenderness, reduce tightness, improve functional capacity, improve mobility.

Subluxations found on assessment and adjusted: C1 right; C3 left; T1, T2, T4, T6 post; Unless otherwise specified, all areas adjusted using: (diversified).

Physical Modalities: bilateral cervical, thoracic, lumbar, pelvis, sacroiliac intersegmental traction, myofascial release, therapeutic stretching (15 minutes).

Plan of Care & Goals

Plan: Try to make it 10 days to two weeks and if so possible final exam next visit or in a few visits.

Short term goals: start on 07/16/2024, bilateral **full spine** by 8 weeks -> decrease pain, decreased tenderness, improved ADL (100%).

Treatment schedule: 1x/week for 1 week(s) .

End of note. Electronically signed by Michael Fillat D.C on 09/27/2024 at 11:29am

Wednesday October 9, 2024 Provider: Michael Fillat D.C

Date of service: 10/09/2024

NPI#: 1801824404

Electronically signed by Michael Fillat D.C on 10/09/2024 at 3:56pm

Subjective

Did pretty well after last. Some pain after a yoga class that lasted a couple days. But overall almost back to all ADLs without too much difficulty. No headaches since last.

Complaints: **primary** -> bilateral **low back**, acute onset immediately at time of injury, pain radiation: no radiation, pain described as: stiff, intensity: mild, level: 1/10; **secondary** -> bilateral **c/t pain and headaches**, acute onset immediately at time of injury, pain radiation: no radiation, pain described as: sore, stiff, intensity: mild, level: 3/10.

Objective

Visual inspection of the patient revealed: -minors sign, no visible pain.

Mental status: Patient appeared: alert, awake, oriented to person, place, time and situation, able to show knowledge of current events, mildly confused.

Myofascial pain/involvement: left & right suboccipital: tightness, tenderness; left & right cervical paraspinal: tightness, tenderness; left & right thoracic paraspinal: tightness.

Spinal tenderness on palpation: none.

Spinal level: Subluxations found C1, C4, T2, T4, T6.

Assessment

Response to care: patient tolerated today's treatment well & without incident.

Diagnosis: M99.03 (Segmental and somatic dysfunction of lumbar region), M54.51 (Vertebrogenic low back pain), M99.02 (Segmental and somatic dysfunction of thoracic region), M54.6 (Pain in thoracic spine), M99.01 (Segmental and somatic dysfunction of cervical region), M54.2 (Cervicalgia), G44.319 (Acute post-traumatic headache, not intractable), F07.81 (Postconcussional syndrome), M79.2 (Neuralgia and neuritis, unspecified), M79.18 (Myalgia, other site).

Diagnostic Procedures & Treatment Rendered

Region & treatment rationale: acute care region(s) -> (bilateral) cervical, thoracic, lumbar, pelvis, sacroiliac; rationale -> reduce pain intensity, reduce pain frequency, reduce tenderness, reduce tightness, improve functional capacity, improve mobility.

Subluxations found on assessment and adjusted: C1 right; C4 left; T2, T4, T6 post; Unless otherwise specified, all areas adjusted using: (diversified).

Physical Modalities: bilateral cervical, thoracic, lumbar, pelvis, sacroiliac intersegmental traction, myofascial release, therapeutic stretching (15 minutes).

Plan of Care & Goals

Short term goals: start on 07/16/2024, bilateral **full spine** by 8 weeks -> decrease pain, decreased tenderness, improved ADL (100%).

Treatment schedule: 2x/month for 2 week(s) final exam next visit.

End of note. Electronically signed by Michael Fillat D.C on 10/09/2024 at 3:56pm

Friday October 18, 2024 Provider: Michael Fillat D.C

Date of service: 10/18/2024

NPI#: 1801824404

Electronically signed by Michael Fillat D.C on 10/18/2024 at 12:44pm

Subjective

Didn't mention to me last visit that she had been having some difficulties with sharp, stabbing pain in her head when she would lie on her side while trying to fall asleep. She did reach out to her neurologist about it and he recommended an MRI. Then last Sunday night into Monday morning she woke with the worst migraine of her life and she has been having a headache all week. Can't think of anything she did that caused the migraine or the head pain to start. MRI is scheduled for Monday.

Complaints: **primary** -> bilateral **low back**, acute onset immediately at time of injury, pain radiation: no radiation, pain described as: stiff, intensity: mild, level: 1/10; **secondary** -> bilateral **c/t pain and headaches**, acute onset immediately at time of injury, pain radiation: no radiation, pain described as: achy, dull, sharp, sore, stiff, intensity: moderate, level: 5/10.

Objective

Visual inspection of the patient revealed: -minors sign, no visible pain.

Mental status: Patient appeared: alert, awake, oriented to person, place, time and situation, able to show knowledge of current events, mildly confused.

Myofascial pain/involvement: left & right suboccipital: tightness, tenderness; left & right cervical paraspinal: tightness, tenderness; left & right thoracic paraspinal: tightness.

Spinal tenderness on palpation: none.

Spinal level: Subluxations found C1, C2, T1, T4, T6.

Assessment

Response to care: patient tolerated today's treatment well & without incident.

Diagnosis: M99.03 (Segmental and somatic dysfunction of lumbar region), M54.51 (Vertebrogenic low back pain), M99.02 (Segmental and somatic dysfunction of thoracic region), M54.6 (Pain in thoracic spine), M99.01 (Segmental and somatic dysfunction of cervical region), M54.2 (Cervicalgia), G44.319 (Acute post-traumatic

headache, not intractable), F07.81 (Postconcussional syndrome), M79.2 (Neuralgia and neuritis, unspecified), M79.18 (Myalgia, other site).

Diagnostic Procedures & Treatment Rendered

Region & treatment rationale: acute care region(s) -> (bilateral) cervical, thoracic, lumbar, pelvis, sacroiliac; rationale -> reduce pain intensity, reduce pain frequency, reduce tenderness, reduce tightness, improve functional capacity, improve mobility.

Subluxations found on assessment and adjusted: C1 right; C2 left; T1, T4, T6 post; Unless otherwise specified, all areas adjusted using: (diversified).

Physical Modalities: bilateral cervical, thoracic, lumbar, pelvis, sacroiliac intersegmental traction, myofascial release, therapeutic stretching (15 minutes).

Plan of Care & Goals

Plan: We were going to do final exam at her Wednesday visit next week. I told her to keep visit on the books and if she is feeling much better from this treatment to push the Wednesday out as far as she can. I would like to get MRI results before making any treatment ending decisions.

Short term goals: start on 07/16/2024, bilateral **full spine** by 8 weeks -> decrease pain, decreased tenderness, improved ADL (100%).

Treatment schedule: 2x/month for 2 week(s) final exam next visit.

End of note. Electronically signed by Michael Fillat D.C on 10/18/2024 at 12:44pm

Wednesday October 23, 2024 Provider: Michael Fillat D.C

Date of service: 10/23/2024

NPI#: 1801824404

Electronically signed by Michael Fillat D.C on 10/23/2024 at 3:35pm

Subjective

Last adjustment got rid of the headache. She had an MRI and they said wnl. She was doing well without headache until she woke with one early this morning so she presented. It has been there all day, but not to migraine level.

Complaints: **primary** -> bilateral **low back**, acute onset immediately at time of injury, pain radiation: no radiation, pain described as: stiff, intensity: mild, level: 1/10; **secondary** -> bilateral **c/t pain and headaches**, acute onset immediately at time of injury, pain radiation: no radiation, pain described as: achy, dull, sore, stiff, intensity: mild, level: 3/10.

Objective

Visual inspection of the patient revealed: -minors sign, no visible pain.

Mental status: Patient appeared: alert, awake, oriented to person, place, time and situation, able to show knowledge of current events, mildly confused.

Myofascial pain/involvement: left & right suboccipital: tightness, tenderness; left & right cervical paraspinal: tightness, tenderness; left & right thoracic paraspinal: tightness.

Spinal tenderness on palpation: none.

Spinal level: Subluxations found C1, C2, T1, T3, T6.

Assessment

Response to care: patient tolerated today's treatment well & without incident.

Diagnosis: M99.03 (Segmental and somatic dysfunction of lumbar region), M54.51 (Vertebrogenic low back pain), M99.02 (Segmental and somatic dysfunction of thoracic region), M54.6 (Pain in thoracic spine), M99.01 (Segmental and somatic dysfunction of cervical region), M54.2 (Cervicalgia), G44.319 (Acute post-traumatic headache, not intractable), F07.81 (Postconcussional syndrome), M79.2 (Neuralgia and neuritis, unspecified), M79.18 (Myalgia, other site).

Diagnostic Procedures & Treatment Rendered

Region & treatment rationale: acute care region(s) -> (bilateral) cervical, thoracic, lumbar, pelvis, sacroiliac; rationale -> reduce pain intensity, reduce pain frequency, reduce tenderness, reduce tightness, improve functional capacity, improve mobility.

Subluxations found on assessment and adjusted: C1 right; C2 left; T1, T3, T6 post; Unless otherwise specified, all areas adjusted using: (diversified).

Physical Modalities: bilateral cervical, thoracic, lumbar, pelvis, sacroiliac intersegmental traction, myofascial release, therapeutic stretching (15 minutes).

Plan of Care & Goals

Plan: We were going to do final exam at her Wednesday visit next week. I told her to keep visit on the books and if she is feeling much better from this treatment to push the Wednesday out as far as she can. I would like to get MRI results before making any treatment ending decisions.

Short term goals: start on 07/16/2024, bilateral **full spine** by 8 weeks -> decrease pain, decreased tenderness, improved ADL (100%).

Treatment schedule: 2x/month for 2 week(s) final exam next visit.

End of note. Electronically signed by Michael Fillat D.C on 10/23/2024 at 3:35pm

Tuesday October 29, 2024 Provider: Michael Fillat D.C

Date of service: 10/29/2024

NPI#: 1801824404

Electronically signed by Michael Fillat D.C on 10/29/2024 at 10:57am

Subjective

One headache again since last. Yesterday, so she came in. Stopped wearing her night guard cause she thinks that might be causing headaches.

Complaints: **primary** -> bilateral **low back**, acute onset immediately at time of injury, pain radiation: no radiation, pain described as: stiff, intensity: mild, level: 1/10; **secondary** -> bilateral **c/t pain and headaches**, acute onset immediately at time of injury, pain radiation: no radiation, pain described as: achy, dull, sore, stiff, intensity: mild, level: 3/10.

Objective

Visual inspection of the patient revealed: -minors sign, no visible pain.

Mental status: Patient appeared: alert, awake, oriented to person, place, time and situation, able to show knowledge of current events, mildly confused.

Myofascial pain/involvement: left & right suboccipital: tightness, tenderness; left & right cervical paraspinal: tightness, tenderness; left & right thoracic paraspinal: tightness.

Spinal tenderness on palpation: none.

Spinal level: Subluxations found C1, C3, T1, T3, T5.

Assessment

Response to care: patient tolerated today's treatment well & without incident.

Diagnosis: M99.03 (Segmental and somatic dysfunction of lumbar region), M54.51 (Vertebrogenic low back pain), M99.02 (Segmental and somatic dysfunction of thoracic region), M54.6 (Pain in thoracic spine), M99.01 (Segmental and somatic dysfunction of cervical region), M54.2 (Cervicalgia), G44.319 (Acute post-traumatic headache, not intractable), F07.81 (Postconcussional syndrome), M79.2 (Neuralgia and neuritis, unspecified), M79.18 (Myalgia, other site).

Diagnostic Procedures & Treatment Rendered

Region & treatment rationale: acute care region(s) -> (bilateral) cervical, thoracic, lumbar, pelvis, sacroiliac; rationale -> reduce pain intensity, reduce pain frequency, reduce tenderness, reduce tightness, improve functional capacity, improve mobility.

Subluxations found on assessment and adjusted: C1 right; C3 left; T1, T3, T5 post; Unless otherwise specified, all areas adjusted using: (diversified).

Physical Modalities: bilateral cervical, thoracic, lumbar, pelvis, sacroiliac intersegmental traction, myofascial release, therapeutic stretching (15 minutes).

Plan of Care & Goals

Plan: Final exam next visit. I explained that she just might be at maximum medical improvement as these headaches may just be from ADLs and not the accident at this point.

Short term goals: start on 07/16/2024, bilateral **full spine** by 8 weeks -> decrease pain, decreased tenderness, improved ADL (100%).

Treatment schedule: 2x/month for 2 week(s) final exam next visit.

End of note. Electronically signed by Michael Fillat D.C on 10/29/2024 at 10:57am

Friday November 8, 2024 Provider: Michael Fillat D.C RE-EXAM

Date of service: 11/08/2024

NPI#: 1801824404

Electronically signed by Michael Fillat D.C on 11/08/2024 at 12:02pm

Subjective

Did well after last. Very happy. Was able to make it to today with minimal issues. Also best she has felt between treatments and since beginning of care.

Complaints: **primary** -> bilateral **low back**, acute onset immediately at time of injury, pain described as: stiff, intensity: mild, level: 1/10; **secondary** -> bilateral **c/t pain and headaches**, acute onset immediately at time of injury, pain described as: stiff, intensity: mild, level: 2/10.

Objective

-percussion.

Visual inspection of the patient revealed: -minors sign, no visible pain.

Mental status: Patient appeared: alert, awake, oriented to person, place, time and situation, able to show knowledge of current events, mildly confused.

Cervical ROM: flexion (active): WNL with no pain; extension (active): WNL with no pain; left rotation (active): WNL with no pain; right rotation (active): WNL with no pain; left lateral bending (active): WNL with no pain; right lateral bending (active): WNL with no pain; flexion (passive): WNL with no pain; extension (passive): WNL with no pain; left rotation (passive): WNL with no pain; right rotation (passive): WNL with no pain; left lateral bending (passive): WNL with no pain; right lateral bending (passive): WNL with no pain.

Thoracic ROM: flexion (active): WNL with no pain; extension (active): WNL with no pain; left rotation (active): WNL with no pain; right rotation (active): WNL with no pain; left lateral bending (active): WNL with no pain; right lateral bending (active): WNL with no pain; flexion (passive): WNL with no pain; extension (passive): WNL with no pain; left rotation (passive): WNL with no pain; right rotation (passive): WNL with no pain; left lateral bending (passive): WNL with no pain; right lateral bending (passive): WNL with no pain.

Lumbar ROM: flexion (active): WNL with no pain; extension (active): WNL with no pain; left rotation (active): WNL with no pain; right rotation (active): WNL with no pain; left lateral bending (active): WNL with no pain; right lateral bending (active): WNL with no pain; flexion (passive): WNL with no pain; extension (passive): WNL with no pain; left rotation (passive): WNL with no pain; right rotation (passive): WNL with no pain; left lateral bending (passive): WNL with no pain; right lateral bending (passive): WNL with no pain.

Myofascial pain/involvement: left & right cervical paraspinal: tightness.

Spinal tenderness on palpation: none.

Orthopedic tests: Cervical compression: negative; Cervical distraction: negative; Shoulder depression: negative; Bechterew's: negative; Ely's: negative; Kemp test: negative; Iliac compression: negative; Milgram's: negative; Patrick's Fabere: negative; Piriformis: negative; Straight leg raise: negative; Yeoman's: negative.

Spinal level: Subluxations found C1, C3, T2, L4, left sacroiliac.

Assessment

Response to care: patient tolerated today's treatment well & without incident.

Diagnosis: M99.03 (Segmental and somatic dysfunction of lumbar region), M54.51 (Vertebrogenic low back pain), M99.02 (Segmental and somatic dysfunction of thoracic region), M54.6 (Pain in thoracic spine), M99.01 (Segmental and somatic dysfunction of cervical region), M54.2 (Cervicalgia), G44.319 (Acute post-traumatic headache, not intractable), F07.81 (Postconcussional syndrome), M79.2 (Neuralgia and neuritis, unspecified), M79.18 (Myalgia, other site).

Diagnostic Procedures & Treatment Rendered

Region & treatment rationale: acute care region(s) -> (bilateral) cervical, thoracic, lumbar, pelvis, sacroiliac; rationale -> reduce pain intensity, reduce pain frequency, reduce tenderness, reduce tightness, improve functional capacity, improve mobility.

Subluxations found on assessment and adjusted: C1 right; C3 left; T2 post; L4 left; left sacroiliac; Unless otherwise specified, all areas adjusted using: (diversified).

Physical Modalities: bilateral cervical, thoracic, lumbar, pelvis, sacroiliac intersegmental traction, myofascial release, therapeutic stretching (15 minutes).

Plan of Care & Goals

Short term goals: start on 07/16/2024, bilateral **full spine** by 8 weeks -> decrease pain, decreased tenderness, improved ADL (100%), reached goal on 11/08/2024.

Treatment schedule: released from care.

End of note. Electronically signed by Michael Fillat D.C on 11/08/2024 at 12:02pm